

Document set QC report

Class IIa medical device · EU MDR · four documents checked against each other · claim tracing, numerical consistency, reference verification and findings by severity

This is a redacted sample. It shows the structure and level of detail of a real deliverable. Client, device and project details have been removed or changed. Ask us and we will send one built around a document set like yours.

1 What was checked, and how

Four documents from one submission, read as a single set rather than one at a time. Most of what a notified body finds is not an error inside a document; it is a disagreement between two of them, and no single-document review can see it.

Element	How it was handled
Documents	Clinical evaluation report, instructions for use, labelling text, and the post-market clinical follow-up plan.
Reviewer	Independent of the writing team. A writer checking their own document set re-reads what they meant rather than what is on the page.
Claim tracing	Every performance and safety claim traced back to the clinical evidence and forward to every other document repeating it. 34 claims traced.
Numerical consistency	Every figure compared across documents, including its rounding. A value given as 88.7% in one document and 89% in another is a claim discrepancy, not a typographical one.
References	Each cited work opened and read against the statement it supports, rather than matched against the citation. 84 references.
Search currency	The literature search re-run to the delivery date, and anything new appraised.

What this is not: a regulatory review. We check that the documents say the same thing, that each claim has evidence behind it, and that each is written to the standard that governs it. Whether the regulatory strategy is right belongs to your qualified person.

2 Findings

Severity	Finding	What was done
Critical	The clinical evaluation report (8.2) states performance across all three acquisition systems. The instructions for use (4.3) exclude system C. One of the two is wrong.	Escalated the same day. Which one is correct is a regulatory decision about the intended purpose, not an editorial one, so it was not resolved in the document. A notified body comparing the two raises a major non-conformity.
Major	Specificity appears as 88.7% in the clinical evaluation, 89% in the labelling text, and “approximately 90%” in the clinical summary.	Escalated. All three describe one result; read together they describe three, and the third states a performance the evidence does not support.

Severity	Finding	What was done
Major	PMCF objective 3 commits to monitoring real-world performance, with no method, no sample and no decision threshold.	Escalated with a drafted replacement. Nothing can satisfy or fail the objective as written, and this is the most common reason a PMCF plan is returned.
Minor	Nine items: one superseded MEDDEV revision cited, two cross-references left pointing at the wrong section after a renumber, inconsistent device nomenclature in the labelling, and five terminology drifts between documents.	Corrected in place, listed individually in the findings log, and visible in the tracked copies.

What was already right, and is reported because it is. The intended purpose is stated identically in all four documents. Every residual risk identified in the clinical evaluation appears in the instructions for use. Every PMCF objective except the third traces to a gap the clinical evaluation named. A quality control report that lists only faults is not a review of the document; it is a list.

3 Reference verification

Every reference opened and read against the statement it supports. This is the slow part of a quality check and the part most often skipped, and it is where a document quietly stops being defensible.

Result	Count
References checked	84
Verified as cited	78
Correct source, wrong page or section	2
Review cited in place of the primary study	3
Retracted since the document was first drafted	1
Not traceable to any source	0

- **The two locator errors** were corrected. The claim is in the source; the pointer was not.
- **The three reviews** were replaced with the primary studies they were themselves citing. A review is evidence of what a field believes, not of what a study found, and an assessor reading a clinical evaluation knows the difference.
- **The retracted paper** was removed and the claim it supported re-evidenced from two other sources. It was not retracted when the document was drafted. Nothing in a document tells you this has happened; only opening the reference does.

The literature search was re-run to the delivery date. Four relevant papers had been published since the original search; all four were appraised and two were added, with the appraisal recorded in the evidence matrix.

4 What is handed over

- Clean copies of all four documents, with the minor findings corrected.
- Tracked copies, so that nothing was changed silently.
- The findings log: every finding with its severity, its location, what was done and by whom.
- The escalation memo covering the three items that are regulatory decisions, each with the options and what follows from them.
- The claim traceability matrix: all 34 claims mapped to their evidence and to every document that repeats them.
- The reference verification log: all 84, with the six discrepancies and their resolution.
- The re-run search strategy, its date, and the appraisal of what it found.
- The originality report, with matched passages annotated.

Pubrica writes and checks the documents; the claims, the intended purpose and the sign-off remain yours, and your qualified person or regulatory lead owns the strategy. Where a choice is regulatory rather than editorial we bring it to you with the options rather than deciding it. Any AI assistance in drafting is declared.

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